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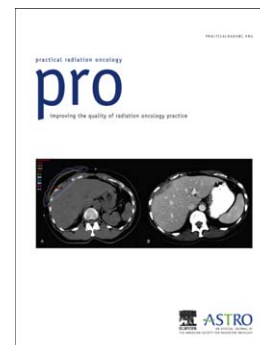
Radiotherapy Quality Assurance & Patient Safety Director Presents: Accreditation Survey Day – Story Experience & Tips

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Title:

Radiotherapy Quality Assurance & Patient Safety Director Presents:

Accreditation Survey Day – Story Experience & Tips

Short Title:

Survey Day – Story & Tips

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Radiotherapy Quality Assurance & Patient Safety Director Presents:

Accreditation Survey Day – Story Experience & Tips

“Uh, oh ...!”

The glass base to my electric toothbrush crashes on the travertine tile between the sink and shower, instantly exploding into a sea of glistening shards. Darn it! My nerves have gotten the best of me.

"Alex! Please bring the broom! ... I'm sorry, but I can't stay to clean up this big mess I made. Can't be late today!" Understanding, he arrives, smiling, calm. Broom in hand. Love him.

One last peek in the long closet mirror. Yes, very professional. White ruffle blouse tucked, black skirt straightened, heels on. Dark striped blazer in hand. Ready to go.

Today feels more like a debutante ball than an accreditation survey. Our “baby” is about to be on display, but also judged. So uncomfortable. We have prepared, but the variables seem infinite. As Quality Assurance and Patient Safety Director of our recently revamped Radiation Therapy Department, all I can think is "What if she trips?" "What if there's a heckler?" You know what I mean.

One deep breath, and I pull myself together. We're ready. Every “i” dotted, every “t” crossed. The nearly 800 pages of recommendations scoured over the last 2 years.¹⁻² But now the moment of truth is upon us. A rectangular beam of sunshine illuminates my keys on the glass kitchen table. I swipe up the keys, hop in the car, and head to work.

The team assembles in the main clinic conference room, a huddle of sorts. Lead Physicist, Lead Therapist, Lead Dosimetrist, Head Nurse, Administrative Director, Clinic Manager, and myself. Necks contort, eyes dart evaluating the situation, last chance for final touches.

We are supposed to have two computers per makeshift surveyor station set up on the conference table and ready to go with access for our guests, but, of course, the computers are not hooked up properly. And the lead IT expert, who has been with us weekly, is unfortunately out sick, today of all days. Murphy's law. But with a call, two emergency computer personnel show up and busy themselves, connecting wires left and right. Yes! A few ties, and the snake pit of wires is neatly arranged. We will be up and running on time.

I press the button on the wall that lowers the projection screen. Not too high, not too low, just right. The pastries on the side table smell buttery and delicious, but my appetite is gone. At least the coffee is hot. I take a 30 second break, stand against the wall between the screen and the food table and my fingers regain some circulation holding the cup. It's coming together.

#

I walk around the department. Last hoorah. I reassure everyone: be open, be honest, let the surveyors know what you do, because you do the right things. Everyone gives me a smile, not sure who is reassuring who. But in fact, I know, we have reviewed every detail, every policy and procedure - updated practices, modified forms, educated, audited, re-trained, re-audited. We're ready.

The surveyors arrive. Nice men. A physicist. A physician. They start with some small talk to warm us up. We go over the group questions. I'm ready for our physicist and lead dosimetrist to argue just a little, the way our normal meetings go when we hash things out. Push, pull. Push, pull. But no, seems the over 100 hours we spent together specifically on this project paid off. All taking turns answering the questions appropriate to their areas, informative and assured. Amazing!

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Time to get down to work. Physicist with physicist. Clinician with Lead Therapist. Seemingly unwelcome, I slink back to my office across the hall. This is where I really feel like I'm mother of the bride. It's out of my hands.

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I peek in. Buttressed by preparatory work, our staff are besides themselves, smiling and directing the surveyors to information in the charts, proud like peacocks. Gone are the frozen wide-eyed stares I remember so distinctively from the beginning of this process. Action has overcome fear. I smile inside.

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Next up: Department Inspection. I follow a few steps behind as our Administrative Director guides the surveyors on a meandering hour-long tour of our staff areas, clinic areas, waiting rooms, equipment and machines. Painless ... if you don't count my heels.

#

Then, dare I say, a seemingly endless private interview with our physics lead. Others have made random excuses to enter the conference room and witnessed the quiet yet

animated chatting between our Lead Physicist and the Physicist Surveyor. Our Administrative Director asks me to check in on them periodically, so I try to pop my head in – but am rebuffed. All seems well. Enough. But there’s nothing I can do. I feel helpless; and hopeful; and confident. Yes, confident. The team really came together in the process of preparation.

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I tell everyone ... It’ll be fine. We had closed the new patient and follow-up clinics to allow for our individual physicians to meet with the surveyors as needed, but we were still treating patients throughout the day. We had to remain focused. And calm. Right?! Right!

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Lunch time. Delicious looking sandwiches, but still, my appetite is gone. I take a diet coke, a sandwich to push around my plate, and a seat at the hard wood oval table across from the surveyors. My only real opportunity to interact with them. The surveyors seem to have been filled in on the role I played in leading and managing the group. I receive firm handshakes, solid looks in the eye, and the general feeling that the surveyors have been impressed with the efforts made to transform our department. The chart reviews have been expedited by a drop-down menu we added to our encounters tab to facilitate their finding of key documents such as H&Ps, Follow-Ups, On-Treatment Notes, and Plans (See Table 1). The surveyors also seem to appreciate the hard copy binders and e-files we created and organized with key policies and procedures, studies, certifications, meeting notes and quality data (See Tables 2 & 3). One of the surveyors is moving up their dinner plans. Yay. The end is near. Just a few more chart checks ...

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4 pm. Time for the summary. Minor suggestions and a warning that we will not really know the outcome for 8-12 weeks. I knew that. It seems like a long time.... But Okay.

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I suggest a pizza party to reward the efforts across the department. But no, our physics lead says “No, ... not until we receive final certificate of accreditation.” Begrudgingly, I go along.

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And then, 10 days after our voluntary site-survey. Surprise! Pizza-time!

References

[1] American Society for Therapeutic Radiology and Oncology.

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https://www.astro.org/uploadedFiles/_MAIN_SITE/Daily_Practice/Accreditation/Content_Pieces/ProgramStandards.pdf. Accessed June 10, 2017.

[2] American College of Radiology.

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<https://www.acr.org/Quality-Safety/Standards-Guidelines/Practice-Guidelines-by-Modality/Radiation-Oncology>. Accessed June 10, 2017.

Table 1.

Unique Encounter Tab Created in Electronic Medical Record to House Key Clinical Documents and Increase Efficiency of Chart Review.

Section Labels – Documents Attached as Relevant*
Clinical
H&P (Radiation Oncology Consultation Note)
Tx Summary (Radiation Oncology Treatment Summary)
FU (Radiation Oncology Follow-Up Visit Notes)
Tx Path (Original Pathology Reports Relevant to Radiation Treatment)
FU Other (Follow-Up Visit Notes from Providers Outside Our Department)
OTV (On-Treatment Weekly Visit Notes While Undergoing Radiation Therapy)
General
H&N (Head and Neck)
Pelvis
CNS (Central Nervous System)
Upper GI (Upper Gastrointestinal)
Breast/Thoracic
SBRT (Stereotactic Body Radiotherapy, Linac or Cyberknife)
Treatment
Tx Plan (Treatment Plan)
IMRT QA (Intensity Modulated Radiation Therapy Quality Assurance)
Electrons (Electron Plan)
CK Plan (Cyberknife Plan)
TBI Plan (Total Body Irradiation Plan)
TSEI Plan (Total Skin Electron Beam Irradiation Plan)
Brachy Plan (Brachytherapy Plan)
MU (Monitor Unit Calculations)
Treatment Checks
Initial Dosimetry Check
Second Physics Check
RTT Pre-Verification (Pre-Treatment by Radiation Therapy Technologist)
Hospital QA (Quality Assurance)
1 st RTT Check
Time Outs
Inside Tx Room (Inside Treatment Room)
Console (Outside Treatment Room)
CK (Cyberknife)
SRS/SBRT (Stereotactic Radiosurgery/Stereotactic Body Radiation Therapy)
Linacs (Linear Accelerators)

* I created a subcommittee of department leaders [Quality Assurance & Patient Safety Director (MD), Lead Physicist, Lead Therapist, Lead Dosimetrist, Lead Nurse, Administrative Manager, Administrative Director, and Information Technology (IT) Expert] to meet weekly, to focus on the accreditation preparation process, and report up to the larger twice monthly departmental

quality assurance meeting, in which all action items were assigned responsible parties, deadlines, and followed through to completion in an excel spreadsheet. Based on self-assessment audits, the subcommittee identified key documents required for review. To increase efficiency and independence of the chart audit process for surveyors, the subcommittee agreed on a list of items to be included in a newly created ARIA[®] encounter menu. Technically, the encounter menu was built by our savvy Lead Therapist. It centrally links to (pulls from) documents housed in various other sections of the electronic medical record. Some documents are scanned in from other electronic medical record systems, such as those used by our dosimetrists and physician colleagues in the main hospital. We have clerical systems for identification and scanning of key documents by staff. Attending radiation oncologists and physicists review the scanned records for accuracy as part of our peer review system.

Table 2.

Hardcopy Binder with Key Documents*

Table of Contents (Including Relevant Policy Numbers, Titles, and Page Numbers within Binder)
List of Physician Privileges
CME Documentation (2 years)
Examples of Initials and Signatures
Emergency Radiation Therapy Treatment Staffing
Competency (Policy/Procedures/Results)
Personnel Orientation Training
Retention of Hospital Records
Follow Up Appointments Procedure
Safety Program/Statement of Authority of Safety Officer
Emergency Management Program
Universal Protocol for Operative/Invasive Procedures
Infection Prevention and Control Plan
Hospital Evacuation Plan
Impaired Communication Interpreter Services
Outcome Studies – Clinical
Focus Studies – Performance Improvement
Morbidity and Mortality Rounds
Chart Rounds/Peer Review

* Based on self-assessment tools, the accreditation subcommittee identified key hospital policies, credentialing data, studies, and meeting minutes that the surveyors would ask to see. The subcommittee decided to print and present these in hardcopy form, rather than requiring real-time staff assistance on survey day. Our Administrative Manager printed the documents and created the hardcopy binder with section tabs listed above.

Table 3.
Electronic Physics Folder

Folders and Subfolders*
Calibration (Methods Used, Inventory of Devices, and Calibration of those Devices)
Logistics (Accredited Laboratories: Forms, Fees, Contracts, Requests, Feedback)
Machine Instruction Manuals
Film Scanners
Software
Chamber and Electrometer Calibration (Inventory, Reports, Certificates, Cross Checks)
Dosimetric Checks of Beam Output
Checks of Multi-Leaf Collimators
Arc Checks
Checks of Computerized Tomography (CT) Simulator
Checks of Cone Beam Computerized Tomography (CBCT)
Phantoms (Respiratory Motion, Dynamic Motion)
Other Quality Assurance (QA) Devices (Used to Check for Flatness, Symmetry, Centering, Clearance, and Parameters Specific to other Machines such as Gamma Knife, Cyberknife and Brachytherapy)
Cross Checks of Barometer/Thermometer Data
Survey Meters (Inventory with Make, Model, Serial Number, Units Calibrated in (mR/hr) Calibration Date, Next Due Date, Physical Location for Each)
Photos
Dosimetry Computer Stations
QA Devices
Simulation and Treatment Machine Quality Assurance – by Machine
Daily QA
Monthly QA
Annual QA
TG-66 CT Simulator QA Review Form
Imaging QA for Linear Accelerators
Acceptance and Testing Reports
Commissioning Reports
Calibration Reports
Safety Checks Patient Set-Up Devices
Machine Service and Maintenance Reports (with Issues/Tests & Solutions/Results by Date)
QA Data Collection Policies, Procedures, and Forms
Brachytherapy
Acceptance Commissioning Report
Hardware and Software Update QA Record
Training for Each Device (with Printed Name, Signature, Position/Title)
External Beam
Acceptance Commissioning Report
Hardware and Software Update QA Record

Training for each device (with Printed Name, Signature, Position/Title)

Periodic Annual Check (CT number by Density of Tissue vs. Vendor Specifications)

General Quality Assurance

Physics Peer Review Program

Physics Policy and Procedure Manual

American Association of Physicists in Medicine (AAPM) Task Group (TG) Reports

Training Documents – Electronic Record, Brachytherapy, Linacs (Attendance Sheets)

Credentials (Dosimetrists/Physicists) – Certification Verification, CME Credits, Resumes

Monitor Unit Checks for External and Brachytherapy

Custom Electron Cutouts [Verification of Cutout Factor by Beam; Electron Output Factor

Guidelines (Instructions/Restrictions)]

* Based on self-assessment tools, our Lead Physicist collected key physics documents and uploaded them to a special folder created within our departmental shared drive. He subsequently assigned the update of subfolders to staff physicists and dosimetrists, based on their role within the department. The Lead Physicist then periodically (at least every three months) reviews all the subfolders, to be certain assigned responsible parties are fulfilling their obligations, and to add new sections as necessary.